Special Needs Co-Parenting Individual Parent Report

Parent completing assessment

Sam Parent

Child

K.

Assessment date

2026-07-30

Assessment version

snc2-2026-1

Needs categories addressed

- Medical Needs
- Educational Needs
- Psychological or Behavioural Needs

Purpose

This individual parent report walks through the same eight-step workflow you completed - understanding the concern, identifying the child's current needs, reviewing professional recommendations, assessing each parent's understanding, evaluating coordination between homes, surfacing areas of agreement and disagreement, choosing practical co-parenting options, and building an action plan. Every finding below is attributed to your own answers. It does not describe your co-parent.

At a glance

Concerns as described by this parent

- Medical Needs: ADHD medication timing needs to be consistent across both homes.
- Educational Needs: IEP follow-through is inconsistent between homes.
- Psychological or Behavioural Needs: Emotional-regulation strategies work in one home more than the other.

Top 30-day priorities

- Medical Needs: Give the medication at the same time every day, weekends included.
- Educational Needs: Get both parents on the school email list.
- Psychological or Behavioural Needs: Agree on the same three coping tools in both homes.

Pathways where coordination is working

- None recorded.

Pathways where coordination needs work

- Medical Needs
- Educational Needs
- Psychological or Behavioural Needs

Pathway scores

Medical Needs

Reported impact: Notable reported impact (4.00 of 5). Understanding across parents: Generally aligned (3.67 of 5). Coordination between homes: Coordination opportunity (3.00 of 5).

Educational Needs

Reported impact: Moderate reported impact (3.00 of 5). Understanding across parents: Coordination opportunity (3.00 of 5). Coordination between homes: Coordination opportunity (2.80 of 5).

Psychological or Behavioural Needs

Reported impact: Notable reported impact (4.00 of 5). Understanding across parents: Coordination opportunity (2.83 of 5). Coordination between homes: Coordination opportunity (2.80 of 5).

Scores reflect the reporting parent's answers and are never findings of fact.

Step 1. Understand the Concern

Medical Needs

Response

concern: ADHD medication timing needs to be consistent across both homes.
duration: 1 to 3 years
urgency: Needs attention within weeks
impact: 4

Finding

For medical needs, the parent describes the concern as: "ADHD medication timing needs to be consistent across both homes.". Duration: 1 to 3 years. The parent reports notable daily impact; the parent expects this to need attention within weeks.

Child impact

Child impact is reported at level 4 out of 5, which the parent describes above.

Suggested

Start the plan from the parent's stated concern rather than any assumption about what is most important.

Action step

Restate the concern in one sentence you and your co-parent can both agree on before moving to solutions.

Educational Needs

Response

concern: IEP follow-through is inconsistent between homes.
duration: 3 to 12 months
urgency: Longer-term planning
impact: 3

Finding

For educational needs, the parent describes the concern as: "IEP follow-through is inconsistent between homes.". Duration: 3 to 12 months. The parent reports moderate impact; the parent frames this as longer-term planning.

Child impact

Child impact is reported at level 3 out of 5, which the parent describes above.

Suggested

Start the plan from the parent's stated concern rather than any assumption about what is most important.

Action step

Restate the concern in one sentence you and your co-parent can both agree on before moving to solutions.

Psychological or Behavioural Needs

Response

concern: Emotional-regulation strategies work in one home more than the other.
duration: 3 to 12 months
urgency: Unstated
impact: 4

Finding

For psychological or behavioural needs, the parent describes the concern as: "Emotional-regulation strategies work in one home more than the other.". Duration: 3 to 12 months. The parent reports notable daily impact.

Child impact

Child impact is reported at level 4 out of 5, which the parent describes above.

Suggested

Start the plan from the parent's stated concern rather than any assumption about what is most important.

Action step

Restate the concern in one sentence you and your co-parent can both agree on before moving to solutions.

Step 2. Identify the Child's Current Needs

Medical Needs

Response

selected needs: Medication schedule, Specialist appointments, Emergency-care plan

other:

Finding

The parent identified 3 current needs in the medical needs pathway.

Child impact

A shorter, prioritised list of current needs helps the child receive the right kind of support in both homes.

Suggested

Confirm this list matches what your treating professionals have identified - that is often where gaps appear between homes.

Action step

Circle the top three needs to focus on in the next 30 days; share that short list with your co-parent.

Educational Needs

Response

selected needs: IEP or Section 504 services, Classroom accommodations, Communication with teachers

other:

Finding

The parent identified 3 current needs in the educational needs pathway.

Child impact

A shorter, prioritised list of current needs helps the child receive the right kind of support in both homes.

Suggested

Confirm this list matches what your treating professionals have identified - that is often where gaps appear between homes.

Action step

Circle the top three needs to focus on in the next 30 days; share that short list with your co-parent.

Psychological or Behavioural Needs

Response

selected needs: ADHD-related supports, Emotional-regulation strategies, Counselling or psychotherapy

other:

Finding

The parent identified 3 current needs in the psychological or behavioural needs pathway.

Child impact

A shorter, prioritised list of current needs helps the child receive the right kind of support in both homes.

Suggested

Confirm this list matches what your treating professionals have identified - that is often where gaps appear between homes.

Action step

Circle the top three needs to focus on in the next 30 days; share that short list with your co-parent.

Step 3. Review Professional Recommendations

Medical Needs

Response

professionals: Paediatrician, Psychiatrist
recommendations summary present: True
in writing: Yes
understanding: 4

Finding

The parent has engaged 2 professionals on the medical needs concern. Written documentation of recommendations: yes. Self-rated understanding of the recommendations: 4 of 5.

Child impact

When both parents work from the same written recommendation, the child experiences one plan, not two.

Suggested

If the recommendation is not in writing, request a summary letter from the treating professional - it removes room for interpretation.

Action step

Save the most recent recommendation as a PDF or photo you can share with your co-parent verbatim.

Educational Needs

Response

professionals: Teacher, IEP or 504 team, School counsellor
recommendations summary present: True
in writing: Yes
understanding: 4

Finding

The parent has engaged 3 professionals on the educational needs concern. Written documentation of recommendations: yes. Self-rated understanding of the recommendations: 4 of 5.

Child impact

When both parents work from the same written recommendation, the child experiences one plan, not two.

Suggested

If the recommendation is not in writing, request a summary letter from the treating professional - it removes room for interpretation.

Action step

Save the most recent recommendation as a PDF or photo you can share with your co-parent verbatim.

Psychological or Behavioural Needs

Response

professionals: Psychologist, Therapist or counsellor
recommendations summary present: True
in writing: Partly
understanding: 4

Finding

The parent has engaged 2 professionals on the psychological or behavioural needs concern. Written documentation of recommendations: partly. Self-rated understanding of the recommendations: 4 of 5.

Child impact

When both parents work from the same written recommendation, the child experiences one plan, not two.

Suggested

If the recommendation is not in writing, request a summary letter from the treating professional - it removes room for interpretation.

Action step

Save the most recent recommendation as a PDF or photo you can share with your co-parent verbatim.

Step 4. Each Parent's Understanding

Medical Needs

Response

score: 3.67

classification: Generally aligned

Finding

On the parent's own answers about how both parents share information and understanding of this concern, the composite is 3.67 of 5 (generally aligned).

Child impact

Aligned understanding between parents reduces mixed messages to the child and to the professionals involved.

Suggested

Schedule a short recurring check-in (email or app) where each parent confirms they have the same information.

Action step

Agree on one shared source of truth for this concern - e.g., a shared folder with the latest reports and instructions.

Educational Needs

Response

score: 3.0

classification: Coordination opportunity

Finding

On the parent's own answers about how both parents share information and understanding of this concern, the composite is 3.0 of 5 (coordination opportunity).

Child impact

Aligned understanding between parents reduces mixed messages to the child and to the professionals involved.

Suggested

Schedule a short recurring check-in (email or app) where each parent confirms they have the same information.

Action step

Agree on one shared source of truth for this concern - e.g., a shared folder with the latest reports and instructions.

Psychological or Behavioural Needs

Response

score: 2.83

classification: Coordination opportunity

Finding

On the parent's own answers about how both parents share information and understanding of this concern, the composite is 2.83 of 5 (coordination opportunity).

Child impact

Aligned understanding between parents reduces mixed messages to the child and to the professionals involved.

Suggested

Schedule a short recurring check-in (email or app) where each parent confirms they have the same information.

Action step

Agree on one shared source of truth for this concern - e.g., a shared folder with the latest reports and instructions.

Step 5. Coordination Between Homes

Medical Needs

Response

score: 3.0

classification: Coordination opportunity

Finding

On how consistently the plan is executed across both homes, the composite is 3.0 of 5 (coordination opportunity).

Child impact

Consistent routines across homes are the single biggest predictor of a child feeling settled while adults resolve differences.

Suggested

Focus first on the smallest routine that is easy to align in both homes (bedtime, morning routine, medication timing).

Action step

Pick one routine to align this week; agree on the exact time, the exact steps, and how to signal when it slips.

Educational Needs

Response

score: 2.8

classification: Coordination opportunity

Finding

On how consistently the plan is executed across both homes, the composite is 2.8 of 5 (coordination opportunity).

Child impact

Consistent routines across homes are the single biggest predictor of a child feeling settled while adults resolve differences.

Suggested

Focus first on the smallest routine that is easy to align in both homes (bedtime, morning routine, medication timing).

Action step

Pick one routine to align this week; agree on the exact time, the exact steps, and how to signal when it slips.

Psychological or Behavioural Needs

Response

score: 2.8

classification: Coordination opportunity

Finding

On how consistently the plan is executed across both homes, the composite is 2.8 of 5 (coordination opportunity).

Child impact

Consistent routines across homes are the single biggest predictor of a child feeling settled while adults resolve differences.

Suggested

Focus first on the smallest routine that is easy to align in both homes (bedtime, morning routine, medication timing).

Action step

Pick one routine to align this week; agree on the exact time, the exact steps, and how to signal when it slips.

Step 6. Agreement and Disagreement

Medical Needs

Response

agree: Medication matters and needs to be given daily.
disagree: Weekend timing and whether to skip on holidays.
gap type: Different priorities

Finding

The parent named specific areas of agreement and disagreement. The gap that best explains the disagreement: different priorities.

Child impact

Naming the type of gap moves the conversation from "you're wrong" to "we have different information / priorities / styles".

Suggested

When you next raise this with your co-parent, lead with the areas of agreement - it is the honest, low-heat starting point.

Action step

In the next conversation, state one thing you agree on before raising the disagreement.

Educational Needs

Response

agree: Our child benefits from the IEP.
disagree: Homework routine.
gap type: Different information

Finding

The parent named specific areas of agreement and disagreement. The gap that best explains the disagreement: different information.

Child impact

Naming the type of gap moves the conversation from "you're wrong" to "we have different information / priorities / styles".

Suggested

When you next raise this with your co-parent, lead with the areas of agreement - it is the honest, low-heat starting point.

Action step

In the next conversation, state one thing you agree on before raising the disagreement.

Psychological or Behavioural Needs

Response

agree: Therapy is helping.
disagree: Screen-time rules and how to react to meltdowns.
gap type: Different parenting styles

Finding

The parent named specific areas of agreement and disagreement. The gap that best explains the disagreement: different parenting styles.

Child impact

Naming the type of gap moves the conversation from "you're wrong" to "we have different information / priorities /

styles".

Suggested

When you next raise this with your co-parent, lead with the areas of agreement - it is the honest, low-heat starting point.

Action step

In the next conversation, state one thing you agree on before raising the disagreement.

Step 7. Practical Co-Parenting Options

Medical Needs

Response

selected options: Shared medication log (paper or app), Both parents added to the patient portal, Pillbox or dispenser used consistently in both homes

other option:

Finding

The parent is open to trying 3 practical co-parenting options for this concern.

Child impact

A short list of practical, agreed options is more useful to the child than a long list of ideas nobody has committed to.

Suggested

Ask your co-parent to select the same options (or add their own) and see where the overlap sits before you propose any of them.

Action step

Pick the two options you would try first and note the exact start date in your shared calendar.

Educational Needs

Response

selected options: Duplicate report cards and school communications sent to each parent, Both parents attend IEP or 504 meetings

other option:

Finding

The parent is open to trying 2 practical co-parenting options for this concern.

Child impact

A short list of practical, agreed options is more useful to the child than a long list of ideas nobody has committed to.

Suggested

Ask your co-parent to select the same options (or add their own) and see where the overlap sits before you propose any of them.

Action step

Pick the two options you would try first and note the exact start date in your shared calendar.

Psychological or Behavioural Needs

Response

selected options: Consistent bedtime and morning routines in both homes, Shared coping-skills toolkit that travels with the child

other option:

Finding

The parent is open to trying 2 practical co-parenting options for this concern.

Child impact

A short list of practical, agreed options is more useful to the child than a long list of ideas nobody has committed to.

Suggested

Ask your co-parent to select the same options (or add their own) and see where the overlap sits before you propose any of them.

Action step

Pick the two options you would try first and note the exact start date in your shared calendar.

Step 8. Create an Action Plan

Medical Needs

Response

top priority: Give the medication at the same time every day, weekends included.
next meeting: Within 2 weeks
professional next: Paediatrician - confirm target timing.
concerns about plan: Sunday-to-Monday transitions can slip.

Finding

The parent identified a single 30-day priority and a proposed timeline (within 2 weeks) for the next co-parenting conversation.

Child impact

A visible, dated plan gives the child (and the professionals supporting the child) a shared target to move toward.

Suggested

Send the plan as a short written proposal rather than asking for a live discussion - it lowers the temperature and gives both parents time to think.

Action step

Draft a two-paragraph proposal now: the 30-day priority, the practical option you will try first, and when you want to review it.

Educational Needs

Response

top priority: Get both parents on the school email list.
next meeting: This week
professional next: IEP team - confirm both parents get notes.
concerns about plan:

Finding

The parent identified a single 30-day priority and a proposed timeline (this week) for the next co-parenting conversation.

Child impact

A visible, dated plan gives the child (and the professionals supporting the child) a shared target to move toward.

Suggested

Send the plan as a short written proposal rather than asking for a live discussion - it lowers the temperature and gives both parents time to think.

Action step

Draft a two-paragraph proposal now: the 30-day priority, the practical option you will try first, and when you want to review it.

Psychological or Behavioural Needs

Response

top priority: Agree on the same three coping tools in both homes.
next meeting: Within 2 weeks
professional next: Therapist - ask for a shared coping-skills handout.
concerns about plan:

Finding

The parent identified a single 30-day priority and a proposed timeline (within 2 weeks) for the next co-parenting

conversation.

Child impact

A visible, dated plan gives the child (and the professionals supporting the child) a shared target to move toward.

Suggested

Send the plan as a short written proposal rather than asking for a live discussion - it lowers the temperature and gives both parents time to think.

Action step

Draft a two-paragraph proposal now: the 30-day priority, the practical option you will try first, and when you want to review it.

This report reflects one parent's own answers to the SA CoParents Special Needs Co-Parenting Assessment. It is an educational planning tool - not a diagnosis of the child, not a finding of fact, not a measure of parental fitness, and not legal advice.